

## CAPITAL MARKETS

# Brookfield's \$1B Healthpeak Stake: REITs Are Selling Control to Buy Time

The joint venture gives Healthpeak liquidity without a dividend cut, but the cost is permanent capital structure change.

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A source-grounded Light Tower Insight. The complete note follows.

Brookfield Asset Management is buying a 49 percent stake in 86 outpatient medical buildings from Healthpeak Properties for roughly \$1 billion. The headline is a joint venture. The economic signal is something else: a publicly traded REIT selling a minority interest in its best assets to avoid selling equity into a discount and to keep its dividend intact without cutting leverage.

The transaction, announced Monday, values the portfolio at approximately \$2.1 billion. Healthpeak retains a 51 percent controlling interest and will continue to operate the buildings. It receives gross proceeds of about \$1 billion. That is not a sale. It is a capital structure trade.

Healthpeak, formed in 1985, owns roughly 700 properties across medical office, life sciences, and senior housing. Its stock has traded at a discount to net asset value for much of the past three years, a condition that makes equity issuance expensive and dilutive. Selling a portfolio outright would trigger capital gains and reduce scale. Selling a minority stake lets the REIT raise cash without either penalty.

The portfolio is 95 percent leased across 5.6 million square feet in 11 states. That occupancy rate is the key underwriting assumption. Brookfield is not buying vacancy risk. It is buying a stabilized income stream with a tenant base that includes health systems and physician groups, which tend to have long lease terms and low turnover. The basis is defensible because the cash flow is predictable.

For Brookfield, the deal fits a pattern. Over the past 18 months, its real estate group has executed about \$32.3 billion in acquisitions and raised \$13.2 billion in new capital. The same day this joint venture was announced, Brookfield and the Canada Pension Plan Investment Board closed the \$5.2 billion acquisition of LXP Industrial Trust. The firm is not making small bets. It is deploying large pools of institutional capital into assets that offer scale, yield, and a clear operating thesis.

Medical office fits that thesis. The demographic tailwind from an aging population is real, and outpatient care is a growing share of healthcare delivery. But Brookfield is not buying the story. It is buying a 95 percent leased portfolio at a basis that produces a current yield competitive with alternative fixed-income and real estate investments. The firm's cost of capital, built on a mix of

institutional equity and private credit, is lower than Healthpeak's public market cost of equity. That gap is the economic engine of the deal.

Healthpeak's constraint is the one that matters for the broader REIT sector. Public market investors have been punishing REITs that cut dividends or issue equity at a discount. Selling a minority stake in a portfolio avoids both outcomes. The REIT gets \$1 billion in cash to allocate toward debt reduction, development, or acquisitions without signaling distress. The dividend stays covered. The balance sheet stays investment grade.

The cost is permanent. Healthpeak has given up 49 percent of the cash flow from its best medical office assets. That cash flow will not return. The joint venture structure is not a bridge loan. It is a capital stack change that lasts as long as the assets are held.

For other REITs facing the same math, the question is whether they have assets that can support this kind of structure. Medical office works because the tenant credit is strong, the leases are long, and the operating complexity is manageable. Office, retail, and even multifamily may not offer the same underwriting clarity. Brookfield is not offering this structure to every REIT. It is offering it to Healthpeak because the assets justify the basis.

The market should test whether other large institutional capital providers follow Brookfield's lead. If Blackstone, KKR, or GIC begin structuring similar minority stakes in REIT portfolios, the signal will be clear: public market equity is too expensive, and private capital is willing to fill the gap at a price. If the deals remain idiosyncratic, then Healthpeak's portfolio quality, not the structure, is the story.

For owners and operators watching from the sidelines, the lesson is narrower. A 95 percent leased medical office portfolio with scale and geographic diversity can attract institutional capital on favorable terms. A 75 percent leased office building with a maturing loan cannot. The bifurcation is not just about asset class. It is about whether the cash flow is predictable enough to underwrite without a discount for uncertainty.

Healthpeak has bought time and preserved its dividend. Brookfield has acquired yield at scale. The

deal is not a vote of confidence in REITs broadly. It is a vote of confidence in this portfolio, this operator, and this basis. That distinction matters because it tells the rest of the market exactly what kind of capital is available and exactly what it costs.

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## SOURCES

— Commercial Observer —

<https://commercialobserver.com/2026/07/brookfield-stake-healthpeak-properties-portfolio/>

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